

Transforming Certification for Better Care

# ABMS CONFERENCE 2026

Learning Together: Member Board Innovations  
in Continuing Certification

# Session Overview

A 3C learning community panel on advancing Member Board continuing certification programs — the metrics are the shared language for that learning

## What We'll Cover

- How community learning identifies new areas for program growth across four Member Boards
- Strengthening compliance with the Standards for Continuing Certification
- How the 3C learning community shaped each board's program design
- Education and quality improvement that enrich the diplomate experience
- Why diplomate engagement drives program success

## Who You'll Hear From

ABMS Committee on Continuing Certification (3C), joined by four Member Boards

- American Board of Urology (ABU)
- American Board of Obstetrics and Gynecology (ABOG)
- American Board of Pediatrics (ABPeds)
- American Board of Physical Medicine and Rehabilitation (ABPMR)

**Format:** each panelist presents in turn, with questions and discussion held until all panelists have presented.

## What You'll Take Away

- Opportunities for improvement in Member Board continuing certification programs
- How a learning community surfaces new opportunities for improvement
- Areas that strengthen diplomate education — and the value of engaging diplomates directly in the improvement process

**Format:** each panelist presents in turn, with questions and discussion held until all panelists have presented



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# Metrics for 3C Evaluation of Member Boards' Continuing Certification Programs

Barbara Sampson, MD, PhD 3C Chair

# Disclosure

- None

# Metrics Overview

How the 3C learning community measures progress against the Standards for Continuing Certification

The metrics are how the learning community sees itself. They translate the Standards into a shared set of measures that show where each Member Board's program stands — and surface the opportunities for improvement the panel will discuss.

01

## What the Metrics Measure

13 metrics for the 19 Standards — 8 structural and 5 measures of program health.

02

## How Boards Report

A phased annual submission builds a common evidence base across all Member Boards.

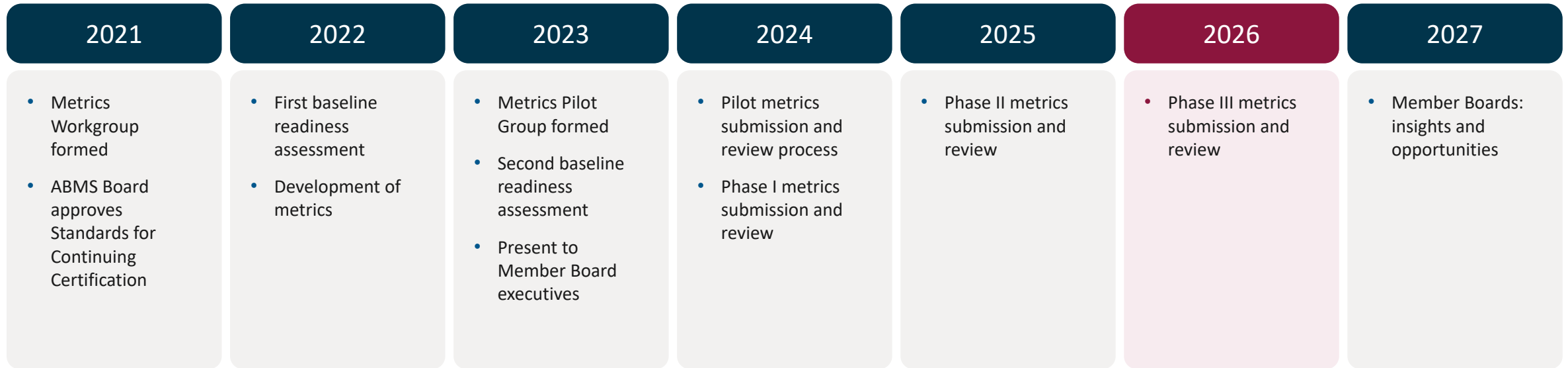
03

## How 3C Reviews

Formative peer feedback — the shared learning the four Boards build on today.

# Standards to Metrics Development

From approved Standards to enduring metrics — a phased, seven-year path



▲ We are here

# Thank You

Sincere thanks for the countless hours your teams dedicated to providing 3C data for the metrics review



**THANK YOU!**

# Metric Development Goals

What the metrics are designed to achieve during the phase-in period

## Assess Against the Standards

Determine whether Member Boards meet the 2021 Standards for Continuing Certification

## Formative, Not Summative

Provide formative recommendations; 3C is not yet making summative compliance decisions

## Establish the Starting Point

Help 3C understand where each Member Board's program begins

## Phase In the Change

Implement a phased approach for a smooth transition to the new metrics and standards

## Learn as a Community

Enable 3C to function as a learning community across Member Boards

## Build Enduring Measures

Establish lasting metrics that define the health of CC programs and support ongoing assessment

# Metrics Development

A 3C Metrics Workgroup designed the metrics that assess how Member Boards meet the new ABMS Standards

13

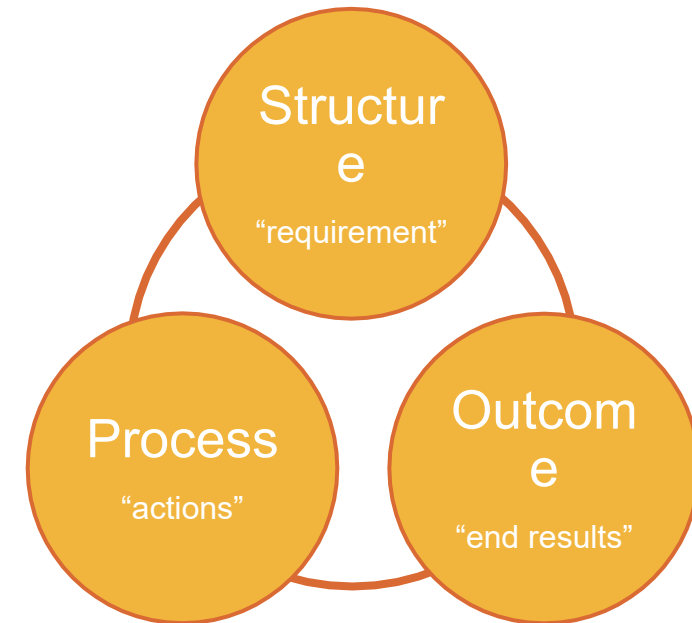
metrics developed for the 19 Standards for Continuing Certification

8

structural metrics — the basic program elements Boards must have in place

5

process and outcome metrics — the ongoing health of CC programs



# Metrics – Structural Metrics

Ensuring ABMS Member Boards have the basic program elements in place to meet continuing certification standards

## Diplomate Notification



Process for Diplomate notification of CC Program

**Std 2, 3, 5, 6**

## Program Evaluation



How CC program is assessed and improved

**Std 7**

## Avoiding Duplication



How duplicative CC requirements are avoided

**Std 8**

## Licensure Review



How licensure review and monitoring is handled

**Std 10, 11**

## Assessment Blueprints



How assessment blueprints are reviewed

**Std 12**

## Knowledge Gaps



How knowledge gaps are addressed, identified, and shared

**Std 15, 16, 17**

2026

**Metrics Under Review in 2026: Quality Agenda Partnerships (Std 18) | Documenting IHHC Participation (Std 19)**



*Completed in 2024 and 2025 – Phase 1 & II Metrics*

# Metrics – Process and Outcome Metrics

Assessing the overall health of continuing certification programs and identifying strengths and opportunities

Phase 2 - 2025

## Metrics Reviewed in This Cycle

### Completion of Cycle

Number of diplomates completing their certification cycle requirements

**Std 3**

### Loss of Certification

Number of diplomates who lost certification and themes for non-completion

**Std 3**

### Regaining Certification

Number of diplomates regaining certification and pathways to reinstatement

**Std 6**

2026

**Metrics Under Review in 2026: Non-Point-in-Time Assessment (Std 13) | IHHC Participation (Std 19)**



Completed in 2024 and 2025 – Phase 1 & II Metrics



# 3C Reviewer Guidelines and Process

How 3C reviewers assess Member Board submissions — objectives and the review process

## Objectives

Foster learning, collaboration, and discussion among committee members

Identify areas of satisfactory compliance

Identify specific areas that need improvement

Provide constructive feedback to each Member Board

## Review Process

### Asynchronous Peer Review

Two independent reviewers assess each submission, followed by a Member Board response

### Reviewer Pool

24 Member Board members, 2 public members, and 2 associate members serve as reviewers

### Oversight and Support

Leadership assists with reconciliation; ABMS staff coordinate calls with Boards for clarification

# Reviewer Rubric

How 3C reviewers assess whether each standard has been met by a Member Board

WAS STANDARD MET: Yes



## Standard Met

The Member Board meets all requirements for this standard.

*No Action Needed*

WAS STANDARD MET: No



## In Progress to Meet Standard

A suitable plan or process is in place, but more time is needed to fully meet the standard.

*Plan Identified*

WAS STANDARD MET: No



## More Information Needed

Additional information is required before a determination can be made.

*Follow-Up Required*

WAS STANDARD MET: No



## Standard Not Currently Met

Some but not all requirements have been met for this standard.

*Action Required*

# Metrics Review Process

Three phases — what your Board does and what 3C does at each stage

## Preliminary

**Your Board**

Submits its metrics data (April – June 2026)

**3C**

Two reviewers complete an independent review and send the preliminary review to your Board for response (June – August 2026)

## Intermediate

**3C**

Reviews all Board submissions together and prepares the draft Metrics Review Report

**Your Board**

Joins a conference call as needed for clarification or additional information

## Concluding

**3C**

Shares the final Metrics Review Report with the ABMS Board of Directors and Member Boards (December 2026)

**Together**

Ongoing monitoring and shared learning across the community (June 2027)

An Oversight Team supports every phase — quality assurance, collaboration, and an equitable process for all Boards

# What We Have Learned

Themes across the 2024 and 2025 metrics reviews — from structural readiness to program performance

## From Readiness to Performance

Phase 1 asked whether boards had the systems; Phase 2 asks how those systems work in practice

## Strong Alignment

13 boards met all standards reviewed in 2024 and six more were one away; most reported 2025 outcome metrics

## Consistency Is the Next Step

Variation in Standards 10, 16, and 17, and in how completion and loss of certification are defined and tracked

## Data Quality in Focus

The question is shifting from “can boards collect the data?” to “can they report it consistently and comparably?”

## Shared Learning Matters

Boards value clearer guidance and want to learn from one another on cycle tracking, data practices, and diplomate support

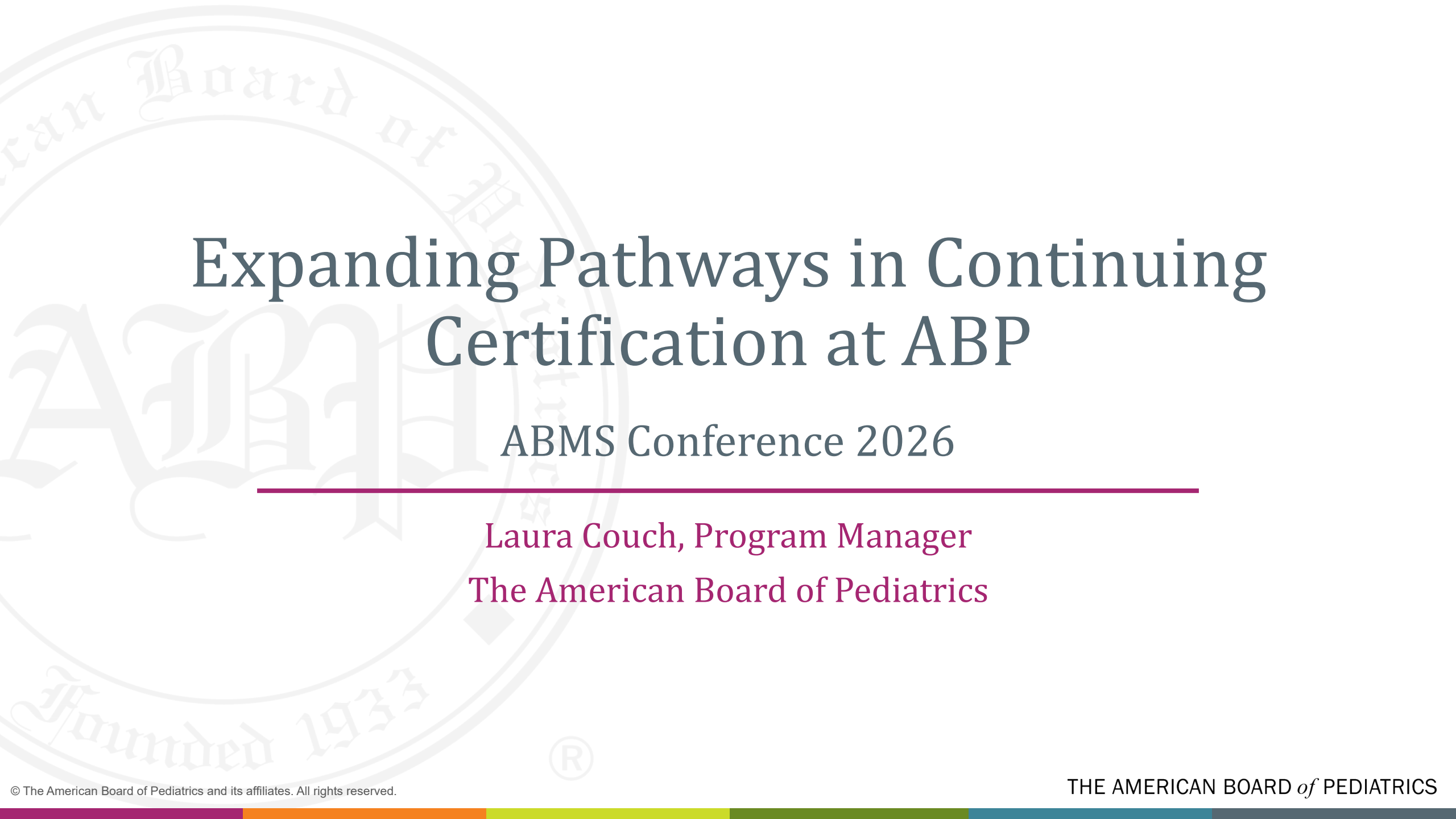
## The Phased Approach Works

Formative reviews and realistic implementation time keep boards engaged and reporting progress

## Toward Program Effectiveness

Phase 3 expands into Standards 13, 18, and 19 and explores broader measures of program impact

Bottom line: the framework is maturing from compliance toward continuous improvement and shared learning



# Expanding Pathways in Continuing Certification at ABP

ABMS Conference 2026

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Laura Couch, Program Manager  
The American Board of Pediatrics

# Disclosures

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*I have no relevant financial relationships with any commercial interests to disclose.*

# Why Change?

## ABP Challenge:

ABP Challenge: Strong Part 4 participation and relevance, but limited pathway options for diplomates whose work falls outside traditional QI.

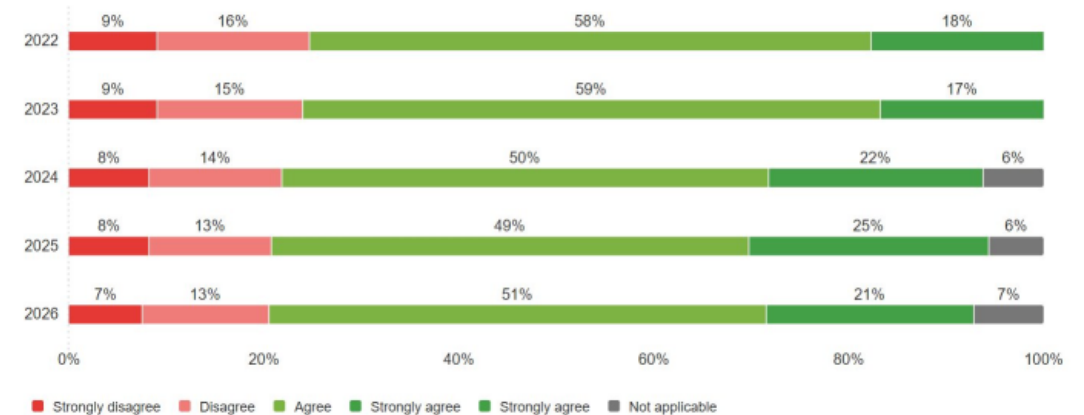
## ABMS Direction:

Expand and maintain relevance by providing meaningful Part 4 opportunities across diverse professional roles and practice settings.

## ABP Response:

Ensure Part 4 activities remain relevant to diplomates across diverse professional roles and practice settings by recognizing improvement work in education (MEPI), community health (PCHI), and research (Physician Scientist Pathway).

Part4\_Eval - Based on your last 5-year MOC cycle, please rate your agreement with the following statement: **The activities I completed were relevant to my practice.**



# Our Approach

- Expanded beyond traditional QI (*Pediatric Community Health Initiative & Medical Education Program Improvement*)
- The ABP began developing the Physician Scientist Pathway nearly 2 years ago and launched it on August 18, 2026.
- Located in ABP Portfolio (login required)
  - Search under Part 4
  - Select Physician Scientist Pathway



# How We Got There

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Discussions with other ABMS member boards to understand how research is incorporated.



Engagement with the pediatric research community (e.g., Society for Pediatric Research and the Pediatric Department Chair Research Committee).



Iterative updates based on feedback from researchers, ABP's Continuing Certification Committee, and ABP Subboards.

# Physician Scientist Pathway

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Recognizes the contributions of pediatric researchers within ABP Continuing Certification Program.



Aligns with updated ABMS Continuing Certification standards.



Fully meets the Part 4 requirement for researchers.



Focuses on those who identify research as their primary career path.



Applies across all types of research (basic science, translational, clinical, etc.).



Includes clear checks and balances so ABP staff can review submissions efficiently.

# Eligibility Criteria

**A pathway for physicians who identify research as their primary career path.**

**The applicant must meet ONE OR MORE of the following criteria:**



**At least 40 percent FTE, averaged over five years, supports research**



**Hold a current federal grant of at least two years duration**



**Research is the primary area of scholarship for academic promotion**



**Have more than 10 publications in peer-reviewed journals over the past five years**

# Application Materials

1

**Two original science or other approved peer-reviewed publications in current MOC cycle**

A PubMed link and PMID number for both publications

2

**A statement describing how the research drives improvements in child health**

3

**An attestation letter from a chair or director who can attest to the research role**

# Physician Scientist Pathway Continued

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## Accepted:

- Original science
- Systematic reviews
- Meta-analyses
- Evidence-based practice guidelines
- AAP clinical reports
- AAP policy statements



## NOT Accepted:

- Commentaries
- Editorials
- Non-systematic review papers
- Book chapters
- Case reports

# A Straightforward, Guided Application

**\*6. What type of organization?**

- ☐ Children's Hospital and/or Academic Medical Center
- ☐ Federal or State Governmental organization
- ☐ Other (not for profit organization)
- ☐ Other (for profit organization)

## Eligibility Attestation

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**7. Please select all that apply (only one required):**

- ☐ FTE Requirement: My research work is  $\geq 40\%$  FTE averaged over the past 5 years.
- ☐ Funding requirement: I hold at least 1 federal grant  $\geq 2$  years duration.
- ☐ Promotion requirement: I have used / will use Research as my primary 'area of scholarship' for academic promotion.
- ☐ Productivity requirement: I have  $> 10$  publications in peer reviewed journals over the past 5 years.

## Personal Statement

### Publication 1

9. Title:

\*10. Journal:

## Research details

\*11. PubMed Link:

12. PMID #:

• **13. PDF Attachment File:**

Please limit the file name to 50 characters and remove any special characters.

# Attestation

## Attestation

\*Please choose one of the following individuals to attest to your research role within your organization, as noted at the beginning of the application.

- ☐ Department Chair
- ☐ Section Chief (Director) / Division Chief (Director)
- ☐ Vice / Associate Chair Research
- ☐ Executive Director Research Institute (or equivalent)

☐ I am a department chair (sign-off waived; will be asked to include a link to your academic/hospital website confirming your role)

## ATTESTATION LETTER

Research Pathway: Improving Health and Health Care

Dear American Board of Pediatrics,

My role in relationship with \_\_\_\_\_ is:

- ☐ Department chair
- ☐ Section Chief (Director) / Division Chief (Director)
- ☐ Vice / Associate Chair Research
- ☐ Executive Director Research Institute or equivalent

I attest that this individual has a primary research role within the department as defined by at least one of the following (check all that apply):

- ☐ FTE Requirement: Research work is  $\geq 40\%$  FTE averaged over the past 5 years
- ☐ Funding requirement: Holds at least 1 federal grant  $\geq 2$  years duration.
- ☐ Promotion requirement: Have used / will use Research as the primary 'area of scholarship' for academic promotion.
- ☐ Productivity requirement: Has  $> 10$  publications in peer reviewed journals over the past 5 years.

I attest that the publications submitted support the overall research mission of the organization.

Sincerely,

# Participation and Early Momentum

Established pathways demonstrate sustained use. The new Physician Scientist Pathway adds a dedicated option for researchers.

## ESTABLISHED PATHWAYS *Launched 1/2024*

Jan. 2024

991

COMPLETIONS

Medical Education  
Program Improvement

Jan. 2024

498

COMPLETIONS

Pediatric Community  
Health Initiative

## NEW PATHWAY

LAUNCHED AUG. 18

Physician Scientist Pathway

24

APPLICATIONS  
SUBMITTED

A dedicated Part 4  
option  
for pediatric researchers

The portfolio now pairs established participation with early momentum in a newly launched research pathway.

# Key Takeaways

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- Role-based vs. project-only participation
- Align pathways with physician career achievements
- Reduced duplication of work
- Flexible but clear criteria
- Efficient review processes



# What We Learned

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One size does not fit all



Clarity + flexibility are both critical



Engagement improves with relevance



Iteration is necessary



Communication and learning from others matter



Maintain rigor in the standard while minimizing application burden

# Opportunities Ahead

*Building on what we learned*



**Better support non-clinical roles**

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**Continue reducing burden**

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**Learn from other ABMS Member Boards**

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**Measure impact vs. participation**

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**Increase meaningful engagement**

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**Align with ABMS standards**

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**Continue expanding relevance while maintaining rigor and reducing burden.**

# THANK YOU!

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**Laura Couch**

[lcouch@abpediatrics.org](mailto:lcouch@abpediatrics.org)

## Explore the Physician Scientist Pathway

Scan the QR code for pathway details.



# Knowledge Exposure

## Incorporating Articles into Continuing Certification

### Challenges/Lessons Learned



**David B. Joseph, MD, FACS, FAAP**  
**Chair-Continuing Urologic Certification**  
**American Board of Urology**

**Professor and Vice Chair**  
**Department of Urology/Heersink School of Medicine**  
**University of Alabama at Birmingham**

# Disclosures

- Lifelong Learning Chair, American Board of Urology

# Goals

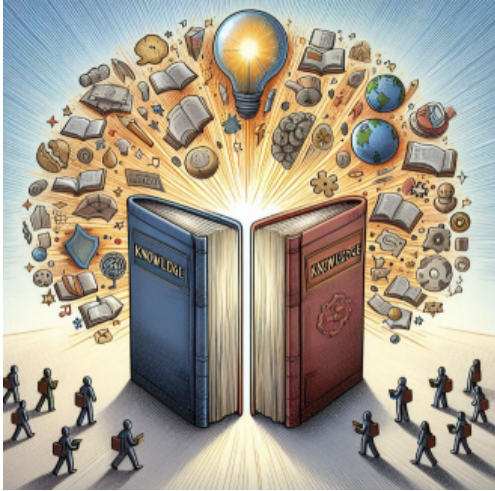
- Review the reading program  
Knowledge Exposure
- Discuss Challenges  
Article availability  
Expense  
Item writing  
Psychometric reliability
- Pivot  
Lesson learned



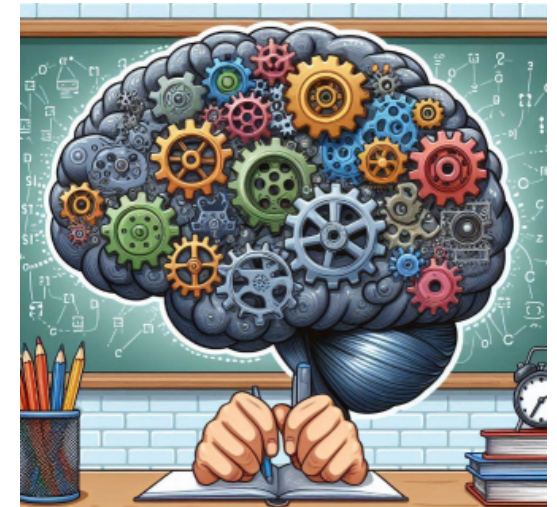
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# CUC 5 Year Cycle

## 3 Learning Activities



- **Knowledge Exposure**
  - new concepts (years 1 & 3)
  - JUROL/UP/JUOP/AUA Guidelines
  - 8 articles (5 questions/article)
- **Knowledge Reinforcement**
  - memory challenge (years 2 & 4)
  - 40 questions
  - personal environment
  - self-paced
- **Knowledge Assessment** (year 5)
  - 90 questions
  - knowledge gaps identified
  - individualized CME remediation



**All 3 activities performance based**



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# Learning Activity Modular Content

- Core
- Oncology, Urinary diversion, Adrenal
- Calculus, Lap-Robotics, Upper tract obstruction
- Impotence, Infertility, Infection, Andrology
- General office-based urology



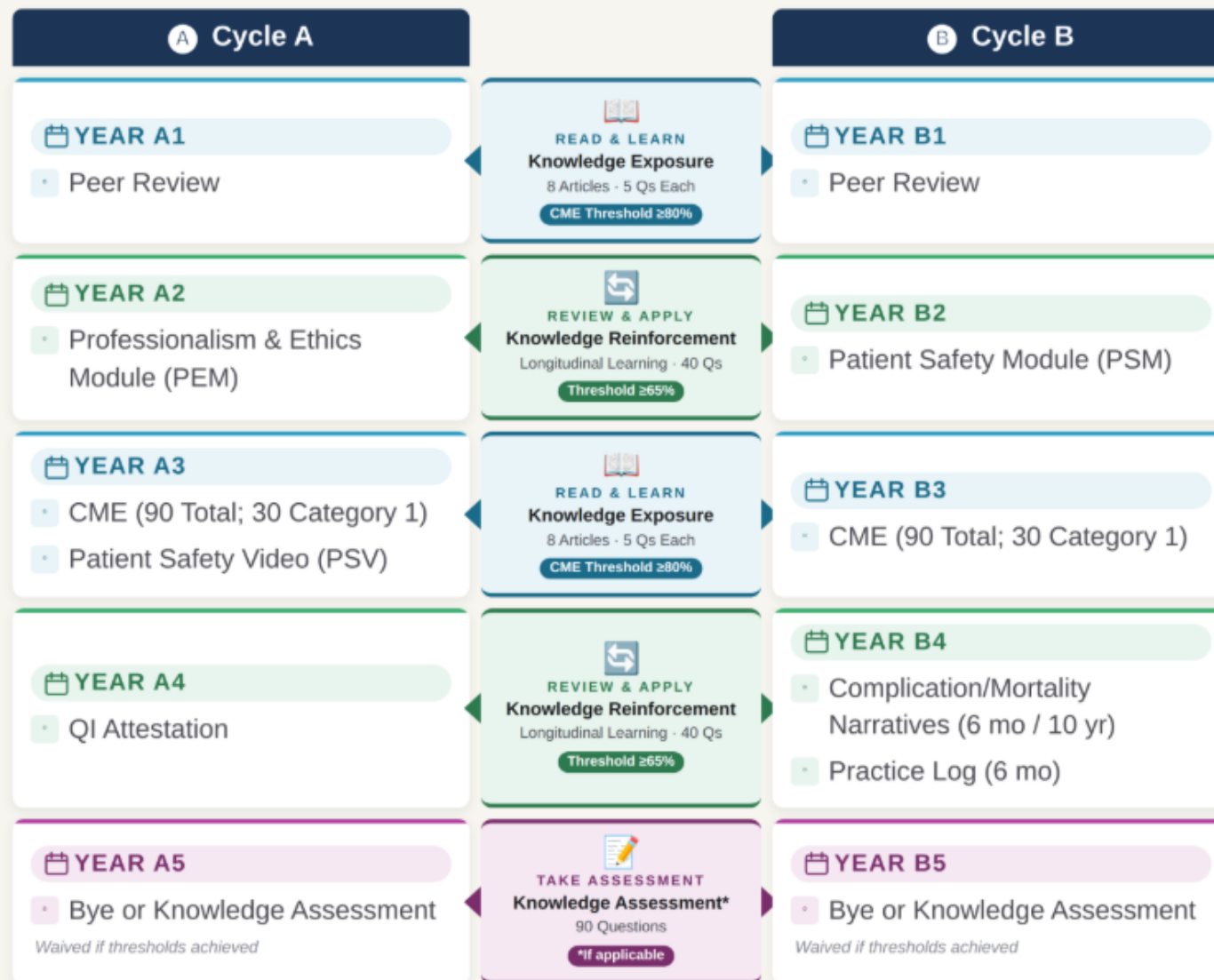
Appreciated that one module does not fit every practice



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# CONTINUING COMPETENCY

## CUC Portfolio 5-Year A/B Cycles

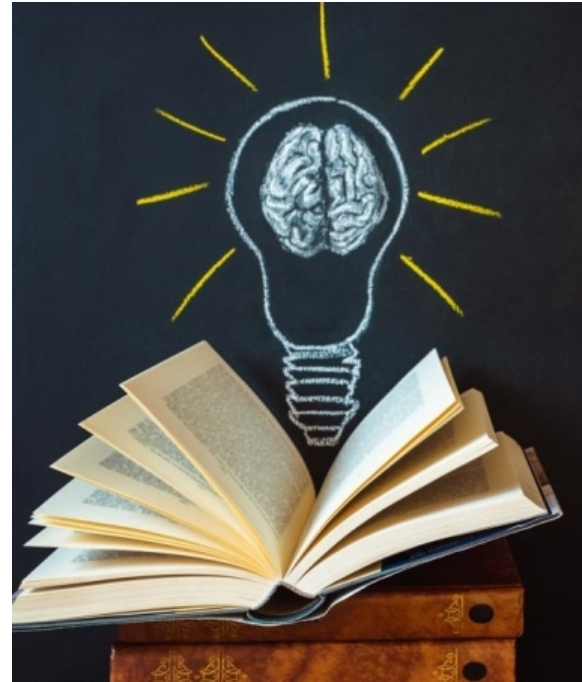


\* **Knowledge Assessment Waiver:** The 90-question KA at Year A5 or B5 is waived if the participant has achieved the Knowledge Reinforcement threshold (≥65%) throughout the respective cycle.

# Knowledge Exposure

New Concepts/Guidelines  
Years 2&4

- Journal of Urology
- AUA Guidelines
- 4 Core
- 4 Module
  - freedom to select
- 5 questions/article
  - ABU requirement



- Sections
  - Core / Guideline
  - Oncology
  - Calculus/Rob
  - Impot/Infert/Andro
  - Office practice
- CME
  - $\geq 80\%$  completion

Partnership with the AUA through the *AUAUniversity*



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## Continuing Urologic Certification (CUC) Learning Group

## Features

Home  
Calendar  
Courses  
News  
Discussions

## ACTIVITY #1 WITH CME CREDIT:

With CME Credit

ACCESS

## ACTIVITY #2 WITHOUT CME CREDIT:

Without CME Credit

ACCESS

## CUC Information

## NEW CUC REVIEW ARTICLE PROGRAM

Welcome to the new AUA 2022 and 2023 Continuing Urologic Certification (CUC) Review Article Pilot Program. This program is only available to ABU Diplomates currently enrolled in the ABU Continuing Urologic Certification program. The goal of this program is to ensure Diplomates are staying current with guidelines and journal articles in their area of practice. As a pilot program, the AUA is offering two versions of the activity – one with CME credit and one without. Each Diplomate is required to select eight articles and complete the associated assessment. The results of each assessment will be provided to the ABU, whether you pass or fail, so please ensure that you take each of the eight assessments seriously.

To begin this program, carefully read the information presented below and then identify which activity you would like to take – Activity #1 with CME credit or Activity #2 without CME credit.

Since this is a pilot program, instructions and information have been shared through the activity, but if you need assistance or have any feedback, please email the Office of Education at [education@auanet.org](mailto:education@auanet.org).

If you have specific questions about ABU's Continuing Urologic Certification (CUC) program, please visit <https://abu.org/learning/continuing-urologic-certification-cuc>.

ACTIVITY #1 CME CREDIT:

This activity will provide 1 hour of CME credit per article with a passing rate of 80%. The total number of CME credit available for Activity #1 is 8 hours. Each article will have a \$15 fee for AUA members and \$25 fee for non-members for a total cost of \$120 member/\$200 non-member. **CME credit will only be available through October 31, 2022.**

After completing this activity, learners will be able to:

1. Demonstrate an increased knowledge base of urology.
2. Apply increased knowledge to improve the quality of patient care.

ACTIVITY #2 NO CME CREDIT:

This activity will provide no CME credit and therefore have no fee associated with it.

You may only attempt each assessment one time. The AUA will not re-set the assessment if the 80% passing rate is not met.

## REQUIREMENTS



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# Article Library

- AUA
  - Partnered with our specialty society
  - Journal of Urology /Urology Practice/Open Urology/Guide
  - Foundation of urology covered

## Subspecialty Certification

- Urogynecology and Pelvic Reconstructive Surgery
  - Neurourology and Urodynamics
    - Wiley publisher
- Pediatric Urology
  - Journal of Pediatric Urology
    - Elsevier



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# Challenges - Expense

- Availability – Open Access
  - Expense to achieve open access
    - per article
    - per diplomate
    - per activity
  - Not comfortable charging an expense for required activity over the ACF
- Action – Open Resource
  - AUA all articles open
    - benefit of membership
    - >95% members
    - Elective CME offered \$15/credit hour (one article)
  - Elsevier and Wiley
    - Both agree to provide articles free of charge
    - >95 % in each specialty a member of their society and have access to journal articles.

# Challenges – Question Content

- Requirement
  - 5 questions/article
  - Content goal
    - 3 directly related to content
    - 2 problem solving based on content
- Action
  - Question development
    - AUA created using resources from CME
    - Elsevier and Wiley
      - Not willing to offer CME
      - Developed specialty item writer
        - Senior writers from exam committee pool
        - Young career urologists (promotion activity)



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# Challenges - Reliability

- Question reliability
  - Quality not to the level of exam committee
  - Not psychometrically validated even if excellent questions
    - Small numbers
    - Multiple articles
      - All diplomates don't select the same articles.
- Action
  - Eliminated the threshold score from KA decision



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# Pivot

- Transitioned Knowledge Exposure
  - From to a performance to participation activity
  - Must answer all questions to receive credit
  - 1st response
- CME provided by AUA on requested basis
  - Must achieve 80% correct – 2 attempts



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# Amplified Continuing Education (ACE) Pilot as a CC Innovation

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Amy E. Young, MD  
ABMS Panel Discussion  
9.17.26

# Disclosures

- Member Board Executive, ABMS
- Distinguished Professor, Dell Medical School



# Why did ABOG embark on this massive shift in its CC program?

- Satisfaction with ABOG's CC program which had been in place since 1985 was high
- Article based assessment with associated CME benefit through ACOG (two-fer) since 1985 and associated performance pathway (2018-2019)
- Gleaned very little actionable information regarding knowledge gaps at the individual and aggregate levels to share with the diplomate or to be a good partner to our professional societies

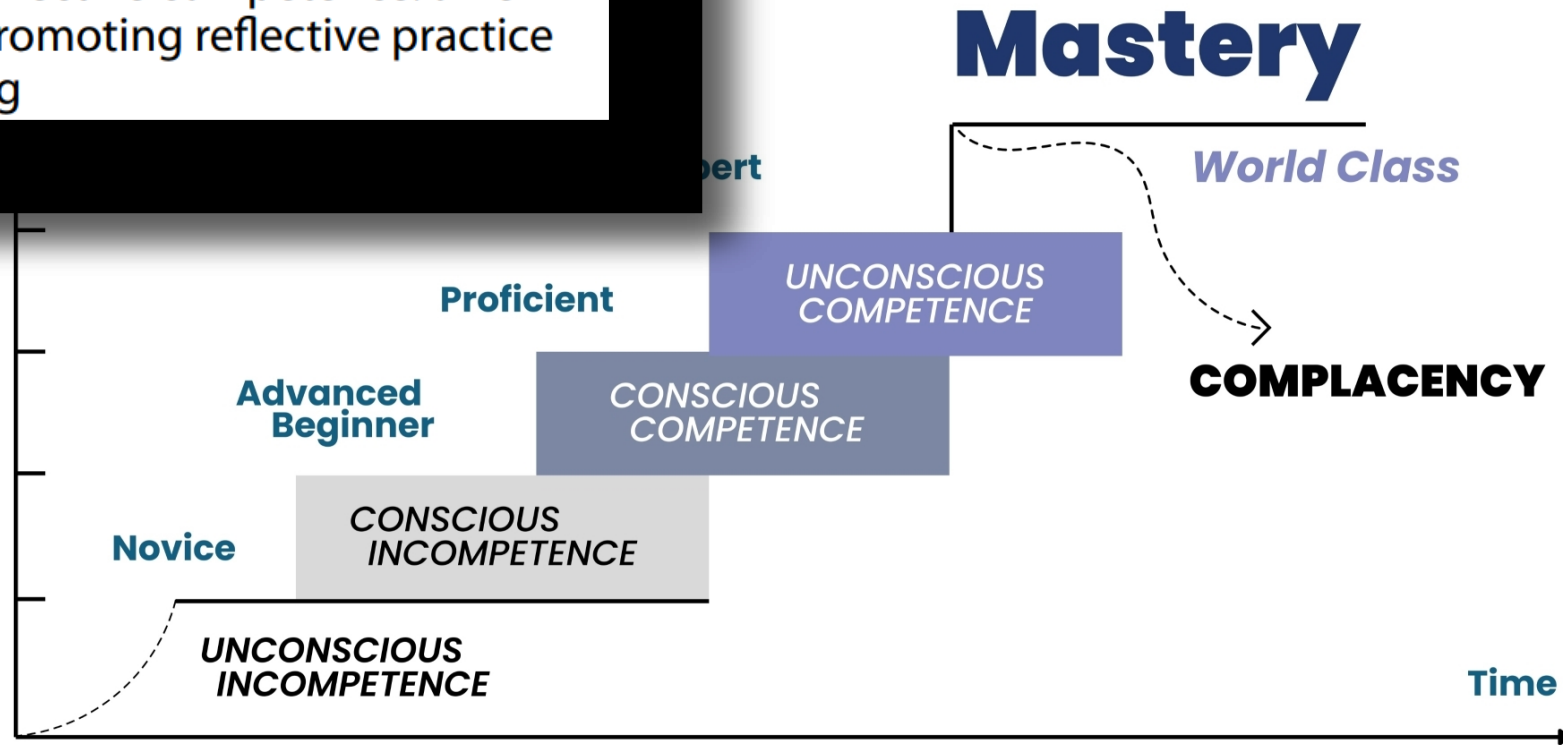
https://doi.org/10.1186/s12909-022-03112-4

RESEARCH

Open Access

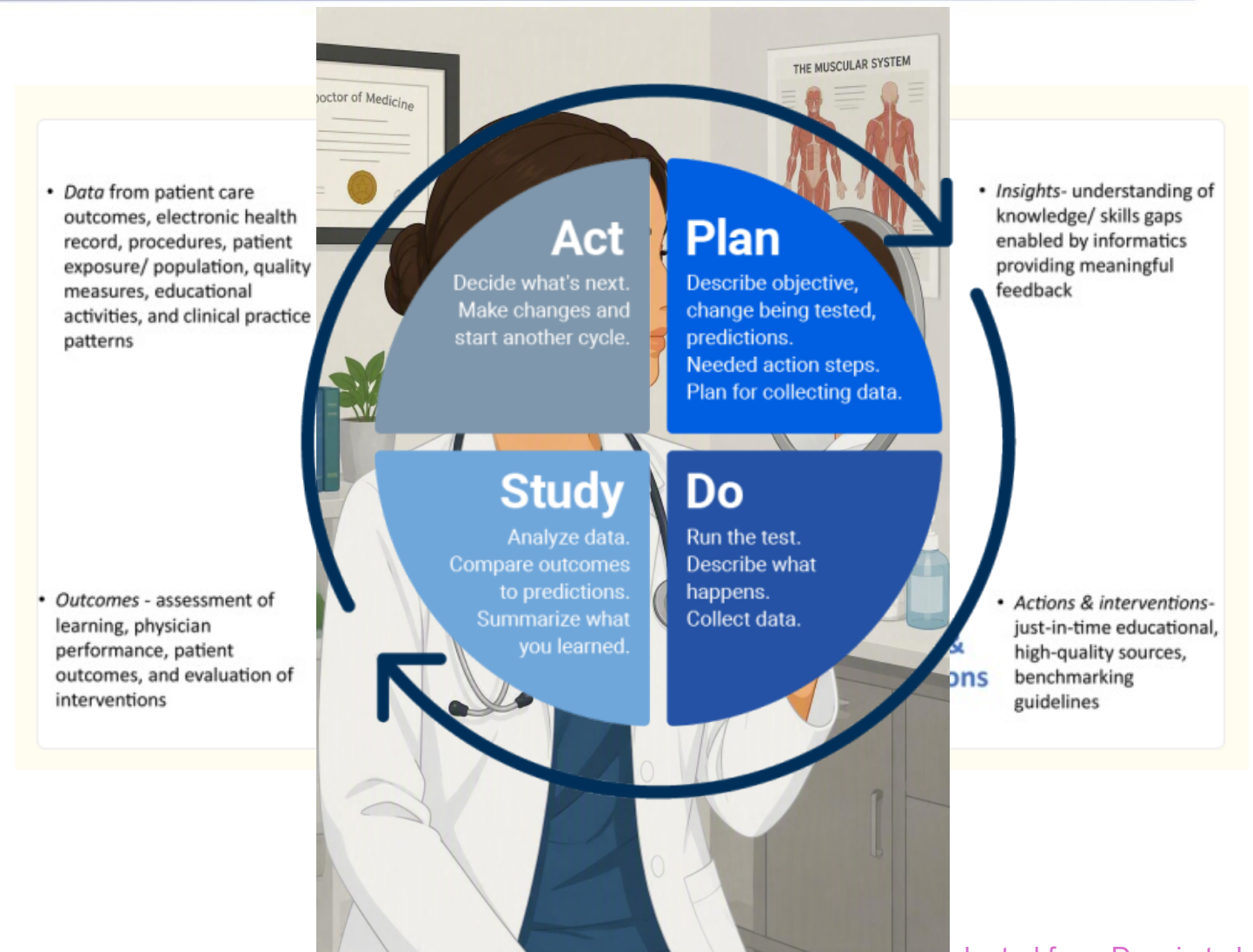


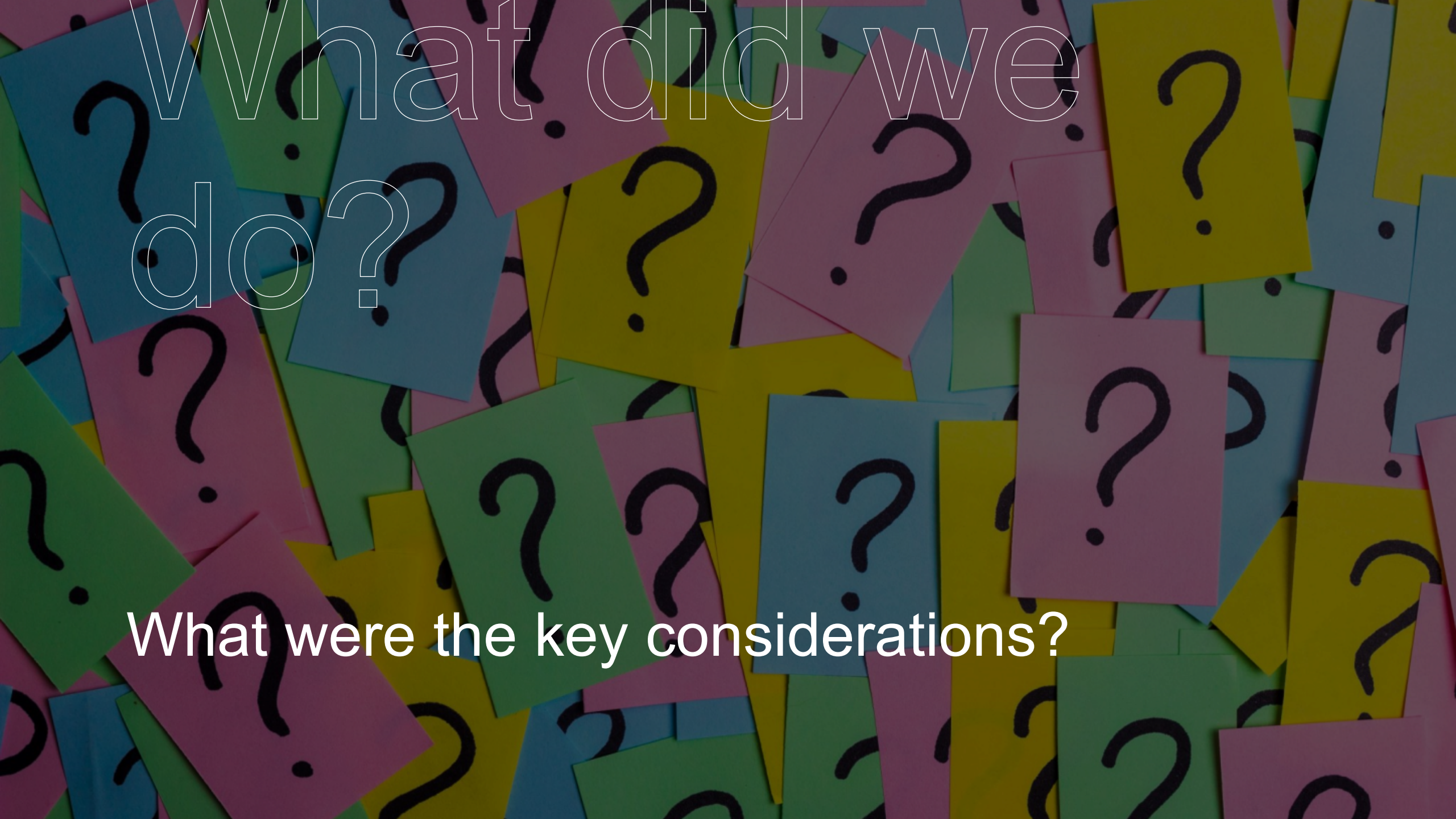
Contextualised reflective competence: a new learning model promoting reflective practice for clinical training



<https://www.frontpage.solutions/blog/four-stages-competence/>

# Shift in Mindset: Programmatic Assessment Using a Precision Education Framework



The background of the slide is a dense, overlapping collage of numerous small, rectangular sticky notes. These notes are in various colors including shades of blue, green, yellow, and purple. Each sticky note features a large, bold, black question mark. The overall effect is one of many questions or uncertainties.

What did we  
do?

What were the key considerations?

# Assessment-Related Standards of Focus

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Balance core content  
with practice-specific  
content



Assess knowledge,  
judgment, and skills



Provide personalized  
feedback to enhance  
learning for diplomates



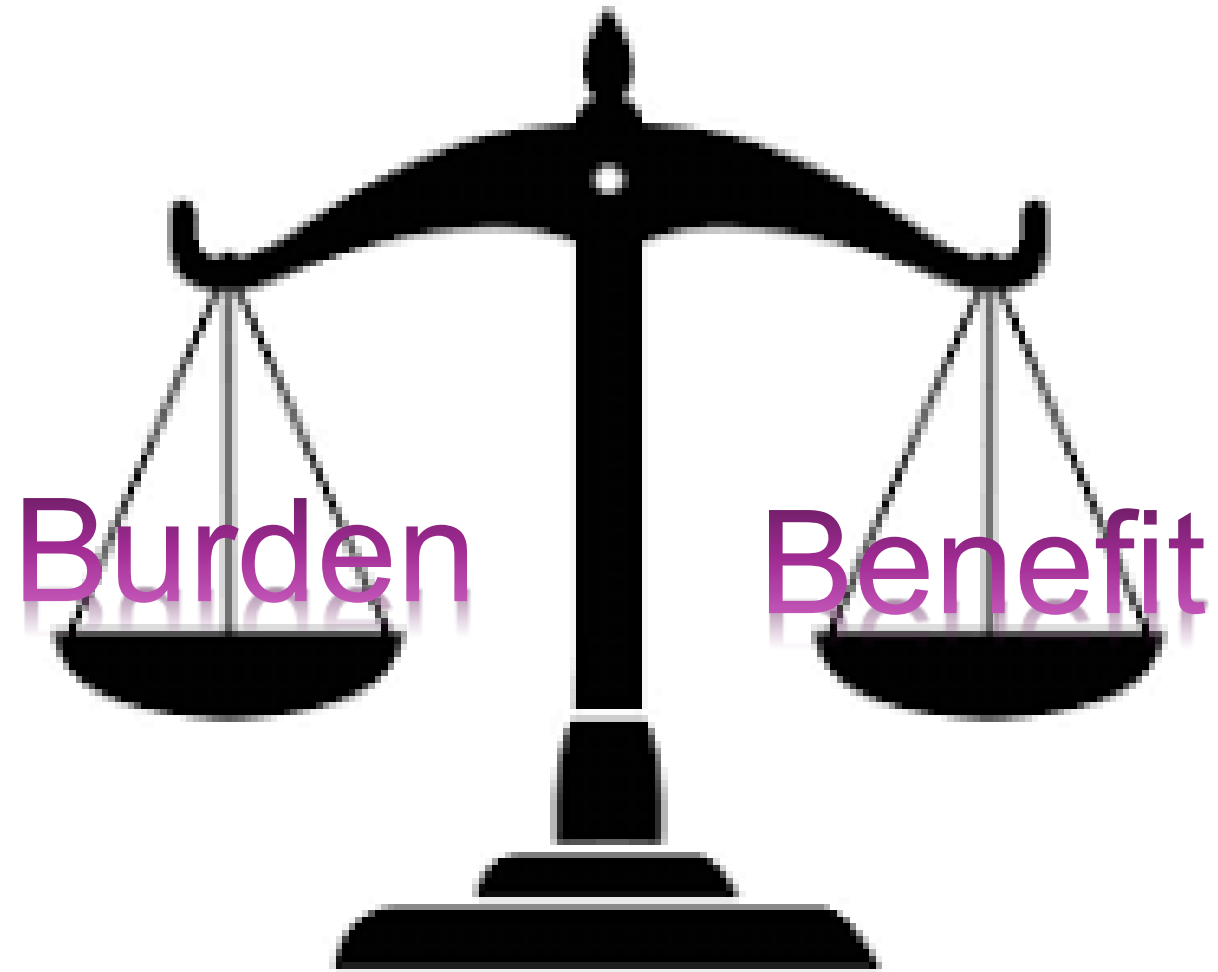
Share knowledge  
gaps with essential  
stakeholders

M

C

V

How to  
balance with a  
new initiative



M

C

V

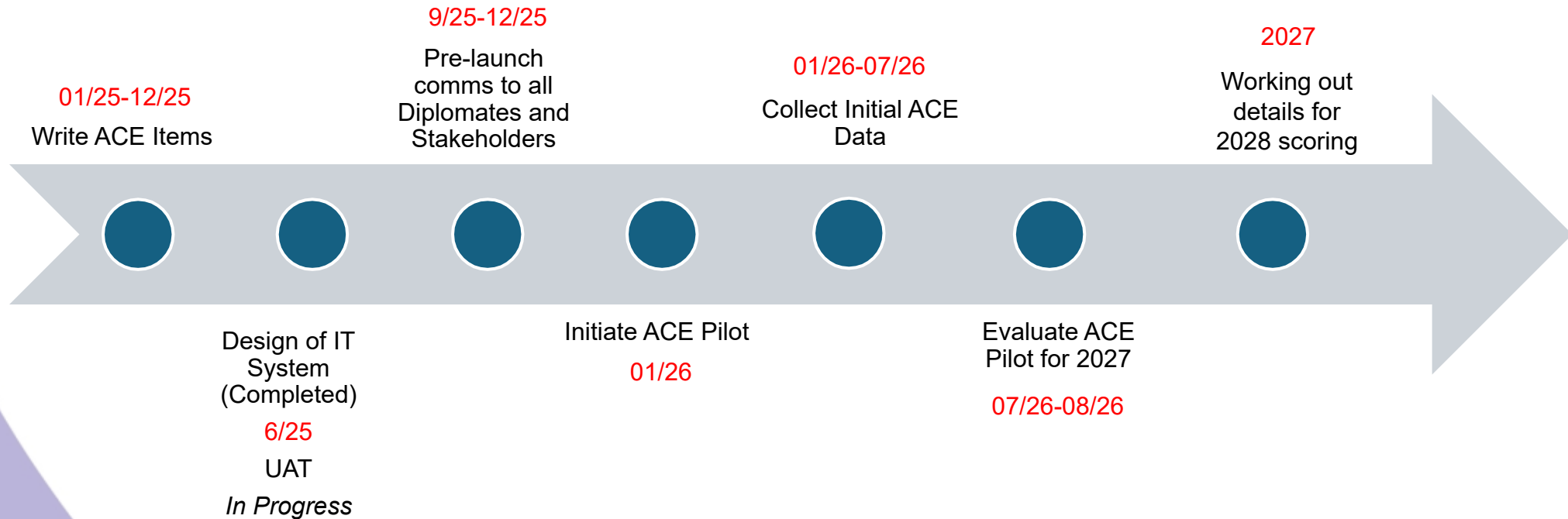
# Panel on Application

## *Area of Practice to Inform Knowledge Gap Questions*

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- Please choose **between 2 to 4** areas of practice (with relative % of focus):
  - ☐ Obstetrics – Labor and Delivery and Inpatient Care
  - ☐ Obstetrics – Outpatient
  - ☐ Gynecology – Surgery (Including Ambulatory Centers) and Inpatient Care
  - ☐ Gynecology – Outpatient
  - ☐ Complex Family Planning
  - ☐ Gynecologic Oncology
  - ☐ Maternal-Fetal Medicine
  - ☐ Reproductive Endocrinology and Infertility
  - ☐ Urogynecology and Reproductive Pelvic Surgery

# Implementation Timeline



M

C

V

Change  
Management  
(on the heels of  
changing the dates  
for the first time in  
20 years)



## Robust Communication Plan

- Created ACE branding/logo
- Key messaging points:
  - Personalized questions
  - No time limit
  - Low-stakes/unscored
  - Immediate feedback
  - Fewer overall questions/minimize burden
- Multimedia approach: 2025 Fall **pre-launch** and 2026 **launch messaging**
  - Targeted emails
  - Social media
  - Quarterly newsletter
  - Website landing/overview page + FAQs
  - Scripted responses for front line teams
  - ACE video distributed via email, shared on social, and shared on website
  - Featured in pre-launch (Sept) Cert60 feature and also in a post-launch (Jan) feature

# EVALUATION PLAN

## How ABOG can judge whether ACE is working

The pilot should be evaluated as both an assessment innovation and a diplomate experience innovation.

### Feasibility

Can the platform, item development, and workflows support the model?

### Relevance

Do diplomates experience ACE as relevant to practice?

### Value

Does feedback support reflection and perceived learning?

### Psychometrics

Do items function well enough to inform future standard setting?

### Knowledge gaps

Can aggregate data identify specialty-level learning needs?

### Usability

Is the diplomate experience manageable and clear?

**For ABMS, the transferable lesson is the evaluation framework: learning value, usability, and measurement quality all matter**



# Early Ace Survey Results (13,000): Not just a pilot...

| Prompts                                                                                                                                                                                                  | Positive<br>(Agree/Strongly<br>Agree) |
|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|---------------------------------------|
| With the addition of ACE Pilot questions for continuing certification, I am able to <b>assess knowledge gaps</b> related to the specialty/my subspecialty                                                | 84%                                   |
| The ACE Pilot questions I received were <b>relevant to my practice</b>                                                                                                                                   | 88%                                   |
| The ACE Pilot questions and the related explanation of the answer choices <b>enhanced my learning</b> in my specialty/subspecialty                                                                       | 88%                                   |
| With the reduction of required articles and the addition of ACE pilot questions (ie, minimal changes), I did not feel an additional <b>burden</b> in completing my continuing certification requirements | 85%                                   |
| With the ACE Pilot questions being unscored, the <b>anxiety</b> I felt in completing these items was low                                                                                                 | 88%                                   |

# ABOG's 2027 Centennial Celebration!





American Board of Physical  
Medicine and Rehabilitation

# **Evolving ABPMR's Continuing Certification Program**

## **ABMS Conference 2026**

Carolyn L Kinney, MD  
Executive Director, ABPMR



American Board of Physical  
Medicine and Rehabilitation

# Disclosures

- Dr. Kinney is employed by the American Board of Physical Medicine and Rehabilitation

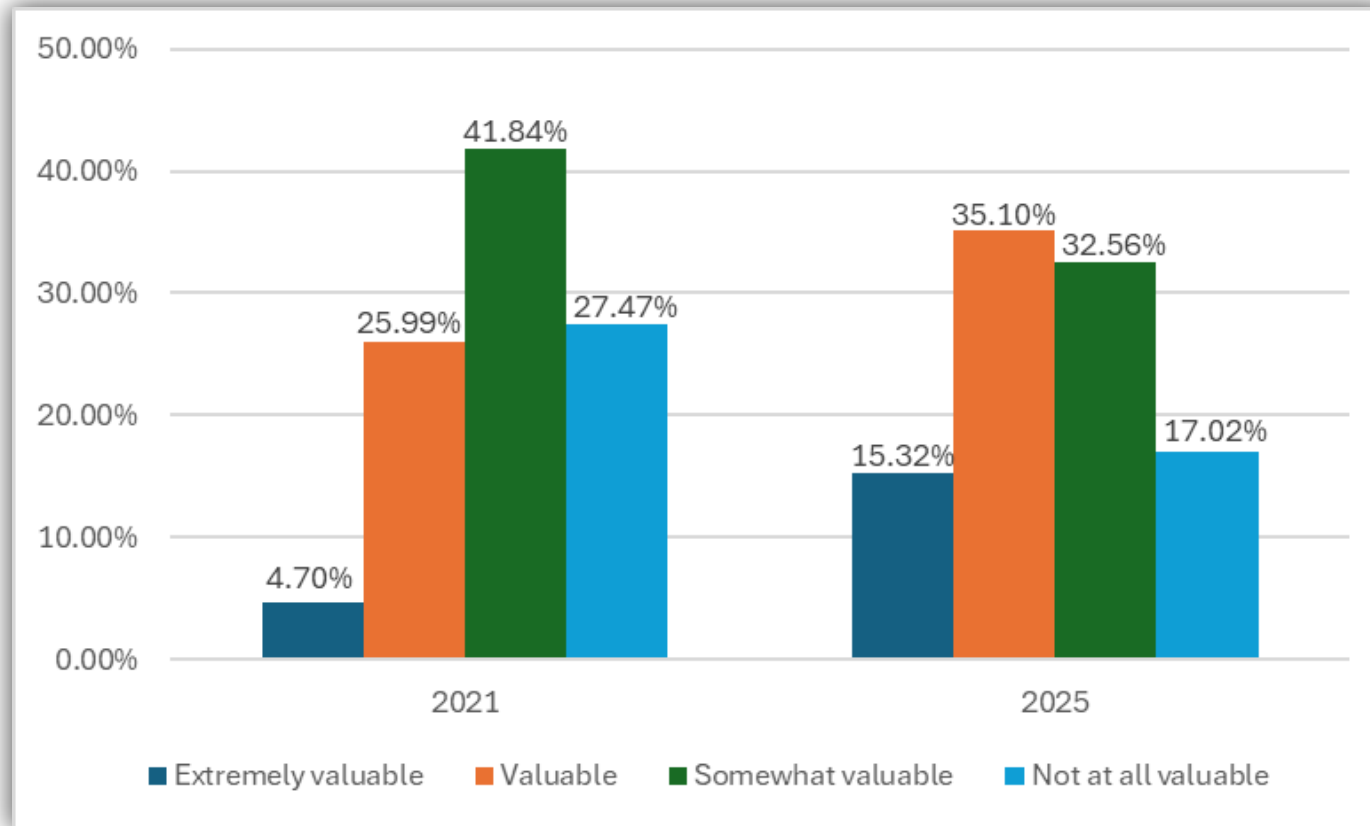


# ABPMR Continuing Certification

- Survey of all diplomates in 2025 with comparison to 2021 survey
- 20% response rate for 2025 survey
- Survey shared with Board of Directors and key volunteers
- Modifications in several areas of Continuing Certification based on diplomate feedback



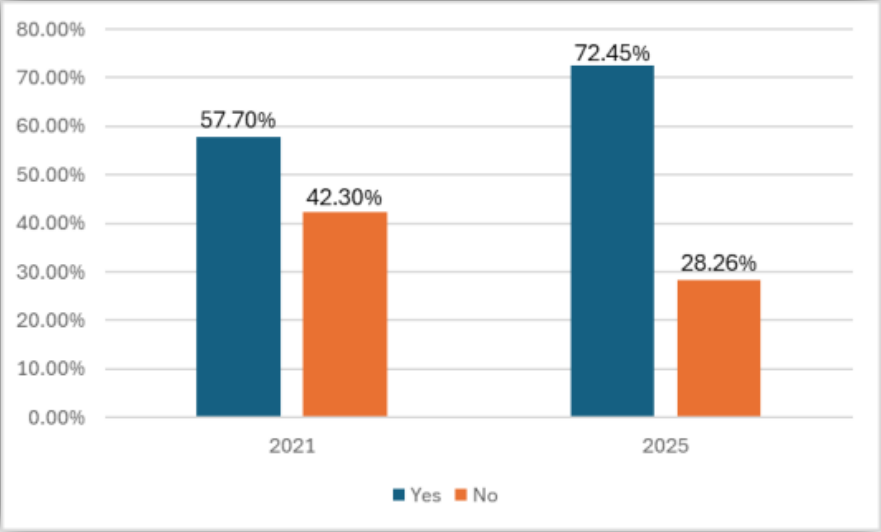
# How valuable have your CC Program activities, including LA-PM&R, been to you and in improving your clinical practice?



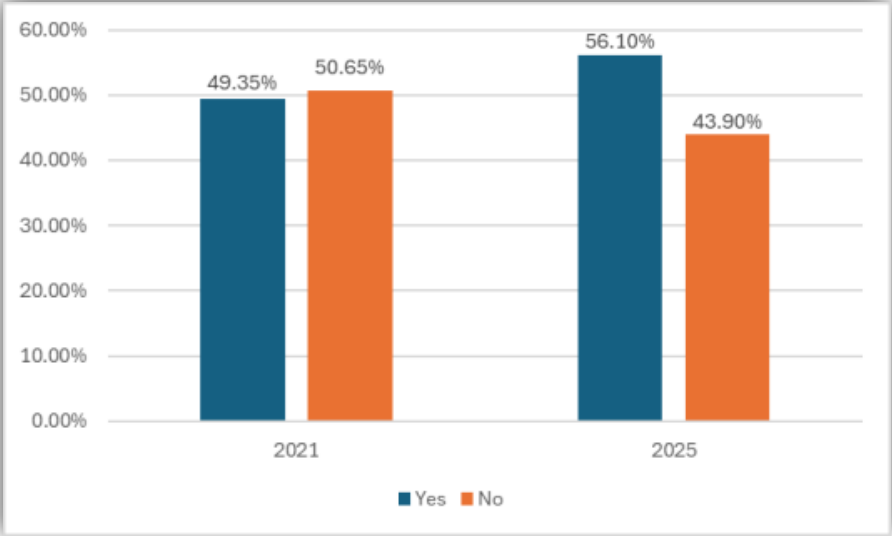
The percentage of positive ratings (valuable/extremely valuable) for the CC program **rose from 30.69% in 2021 to 50.42% in 2025**, representing a 64% relative growth in perceived value.



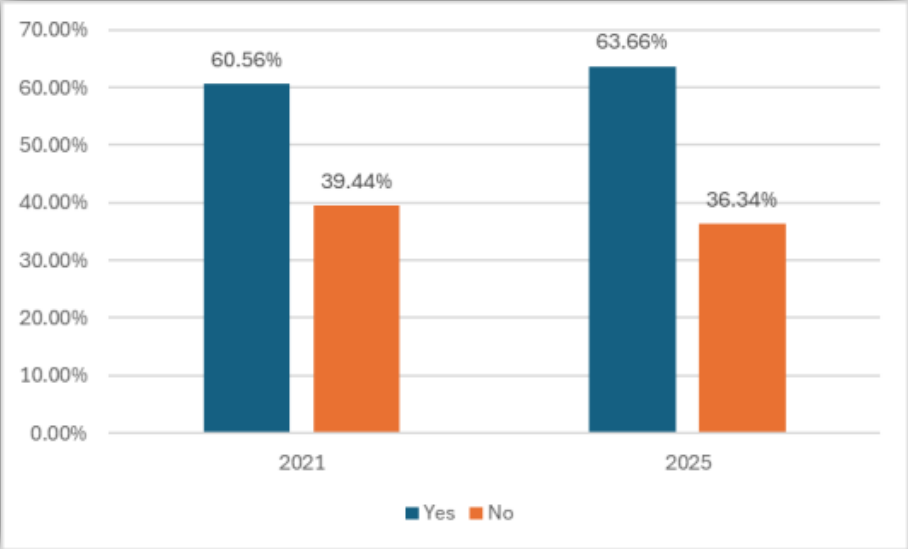
If you used one of the ABPMR QI/IHHC guided templates (the specific topics available via the ABPMR form), was it educational and valuable to you?



Did completing an IHHC project result in an improvement to your practice or in a process?



If you designed/created your own IHHC project, was it educational and useful to



From 2021 to 2025, the percentage of positive (Yes) responses increased for every question.

The most significant improvement is seen in the perceived value of ABPMR IHHC guided templates.

# Continuing Certification Feedback

## Shift in Feedback Themes (2021-2025)

- In 2021, the top theme was "Eliminate QI/PIP."
- By 2025, that request had become a minor theme, while "Reduce burden/time" and "Simplify" emerged as the top themes

## Overall Trend:

Diplomates have moved toward wanting to streamline and improve the CC experience, with emphasis on:

- Reducing burden
- Clearer CME/Self-Assessment pathways
- Greater practice relevance and customization



# ABPMR Response

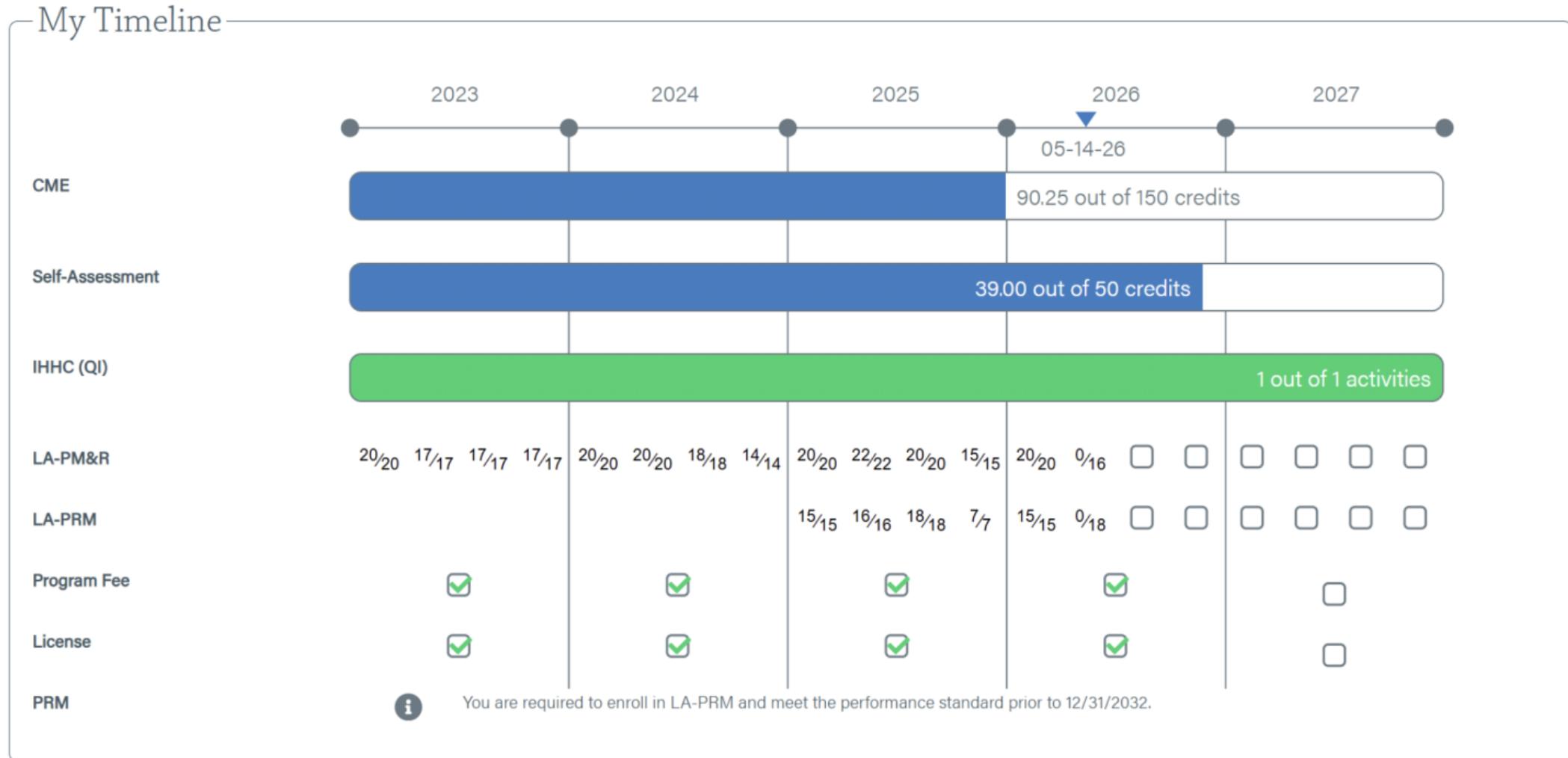
- Platform navigation design
  - Clear “snapshot” of status and outstanding requirements
  - Ability to review previous quarter performance and rationales
- IHHC options
  - Introduced an article-based pathway on quality and safety.
    - Diplomates can opt to participate over 4 years of cycle for IHHC credit. Cannot participate in consecutive cycles
  - Recognized many new options for IHHC credit, including Joint Commission Certification, CARF accreditation, specialty society posters on quality and safety
- Reduced SA-CME requirement to match credits granted for full participation in LA-PM&R



# ABPMR Response

- Increased time for response to LA-PM&R items from 2 to 3 minutes
- Changed scoring of LA-PM&R to emphasize formative intent of the assessment. Diplomates can gain credit if incorrectly answered items are answered correctly in subsequent quarters
- Added articles to Q4 of LA-PM&R that can apply to both primary PM&R certification and subspecialty certification in SCI, BIM and PRM. Increased number of articles available allowing diplomates to focus on areas of practice or interest.
- Website enhancements for all aspects of CC program, targeted email communications, social media engagement

# Platform Navigation

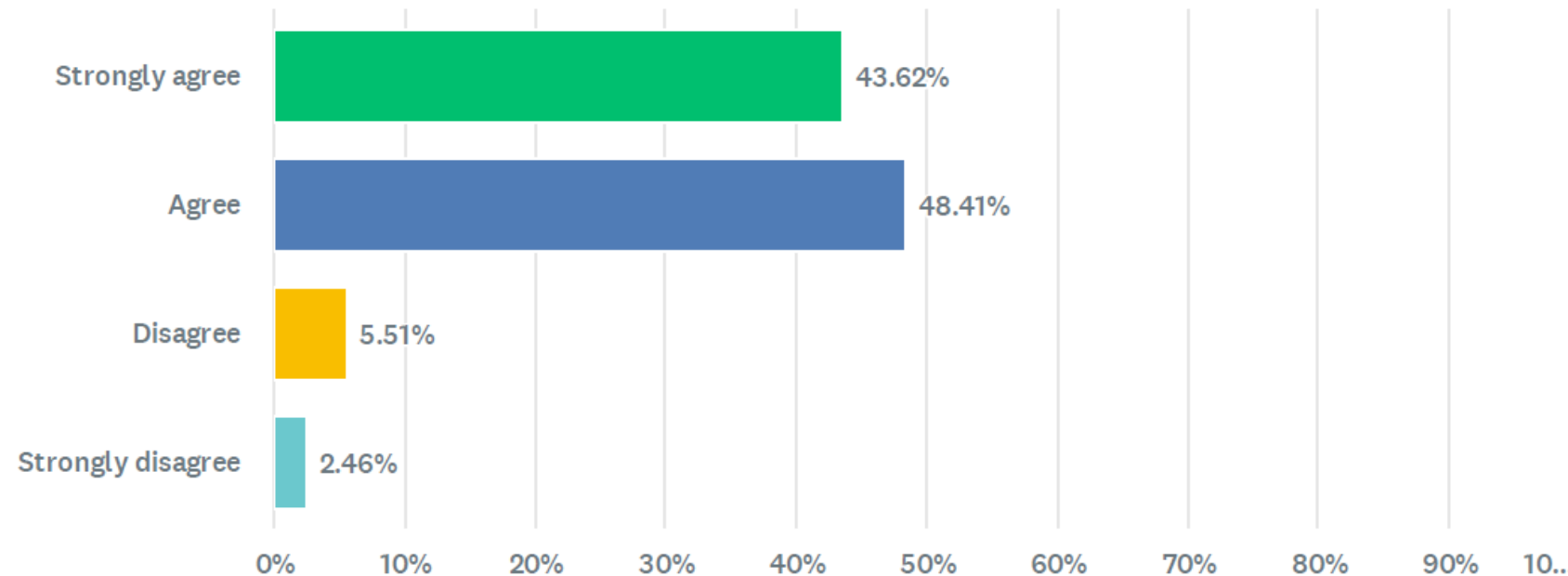


# Platform Navigation



## Q16 The Certification Dashboard is helpful to me in keeping track of my certification requirements.

Answered: 2,359 Skipped: 18



# ABPMR provides the articles, increased options

Select 5 Articles for LA-PM&R 2026

Include in your Assessment?

Recent Advances in the Treatment of Spinal Cord Injury

[Open article in new tab](#)

YES

NO

Long-term Outcomes After Periprocedural and Spontaneous Spinal Cord Infarctions: A Population-Based Cohort Study

[Open article in new tab](#)

YES

NO

Cognitive Motor Dissociation in Disorders of Consciousness

[Open article in new tab](#)

YES

NO

Traumatic Brain Injury and Dementia: Mechanisms, Risk Stratification, and Clinical Management

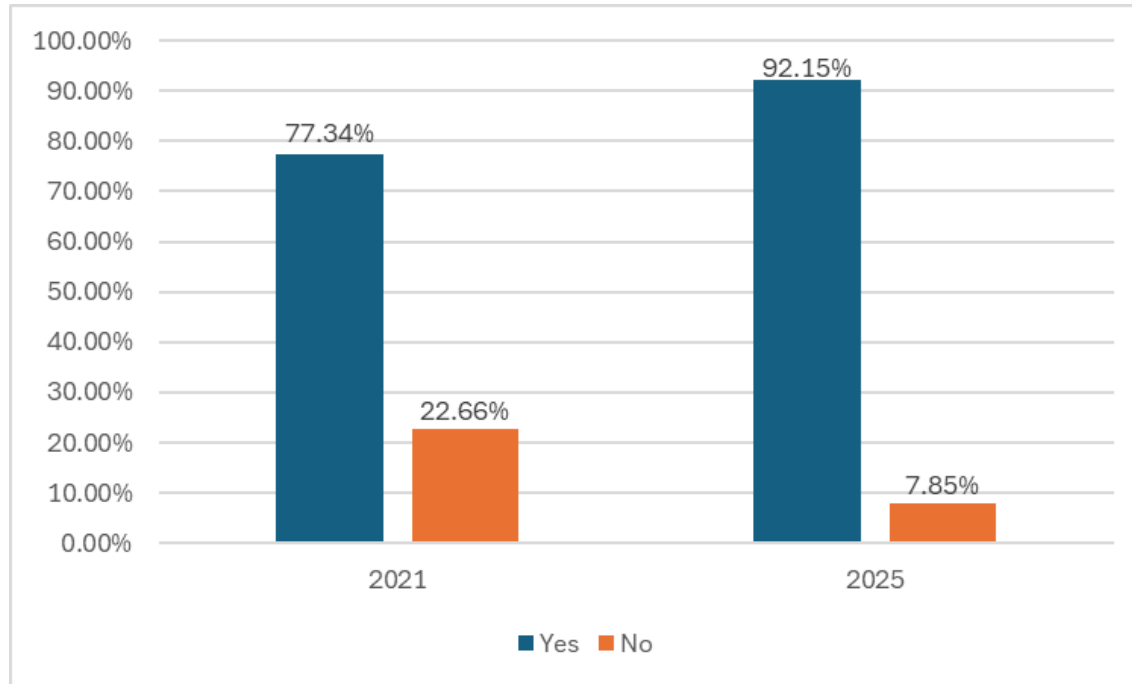
[Open article in new tab](#)

YES

NO



# In your opinion, have the changes the ABPMR has made to the CC program in the last several years improved the program?



- Diplomates recognize progress: Positive ratings (Yes) for the CC program **rose from 77.34% in 2021 to 92.15% in 2025!**
- **2021 appreciation** focused on removing the 10-year MOC exam and adding LA-PM&R
- **2025 appreciation** highlights reattempt credit, article-based questions, and expanded IHHC options
- **2025 Only:**
  - Burden/stress
  - Relevance to practice
  - Confusion related to requirements, navigation and rules
    - Dashboard, notifications, changing expectations





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# Thank You!



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Transforming Certification for Better Care

# ABMS CONFERENCE 2026

## Questions